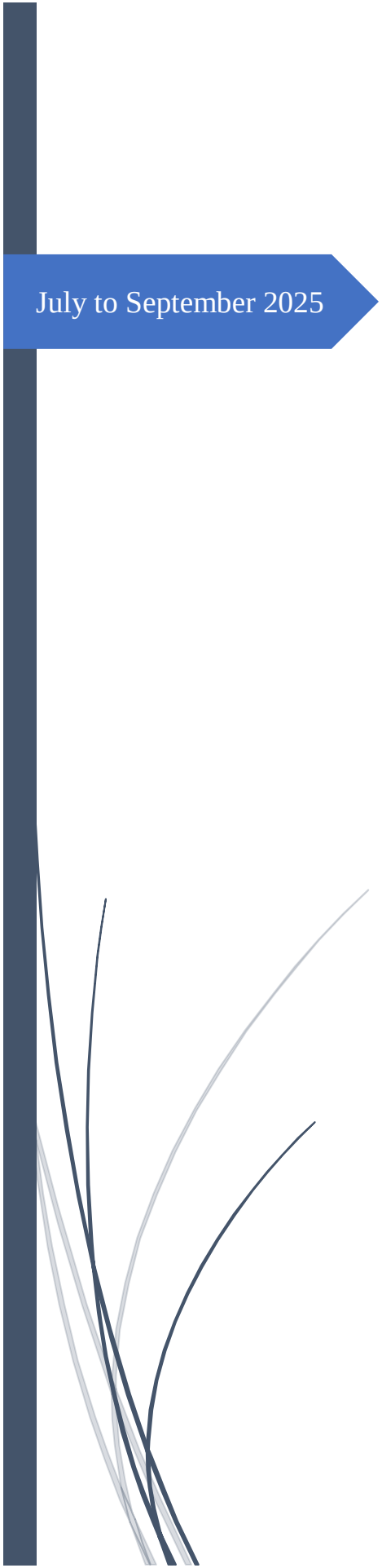




July to September 2025

STI DATABASE MONITORING REPORT

HIV/STI/TB UNIT
STRATEGIC INFORMATION COMPONENT

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Overview

The STI database captures monthly reports on sexually transmitted infections (STIs) submitted by the four regional health authorities across Jamaica. Each region reports based on the number of health centres within its jurisdiction that offer STI testing and treatment services. As at the end of September, the expected number of reports were: Western Region – 162, North East Region – 117, Southern Region – 243, and South East Region – 270, yielding a national total of 792 expected reports. For the July to September 2025 period, the database shows that while national reporting completeness remained high across most regions, timeliness of submissions continued to be a challenge, particularly in the South East Region, where on-time reporting consistently fell below 10%.

Timely and complete reporting is essential for maintaining the accuracy of national surveillance data and underpinning evidence-based decision-making and policy development for STI management. The database monitors trends and occurrences of STIs using key indicators such as cases by sex and age group, genital ulcer disease, genital discharge syndrome, syphilis, epidemiological treatment of sexual partners for chlamydia and gonorrhea, ophthalmia neonatorum (in infants under one month), epididymo-orchitis, pelvic inflammatory disease (PID), genital warts, pediculosis, scabies, other STIs, and HIV testing.

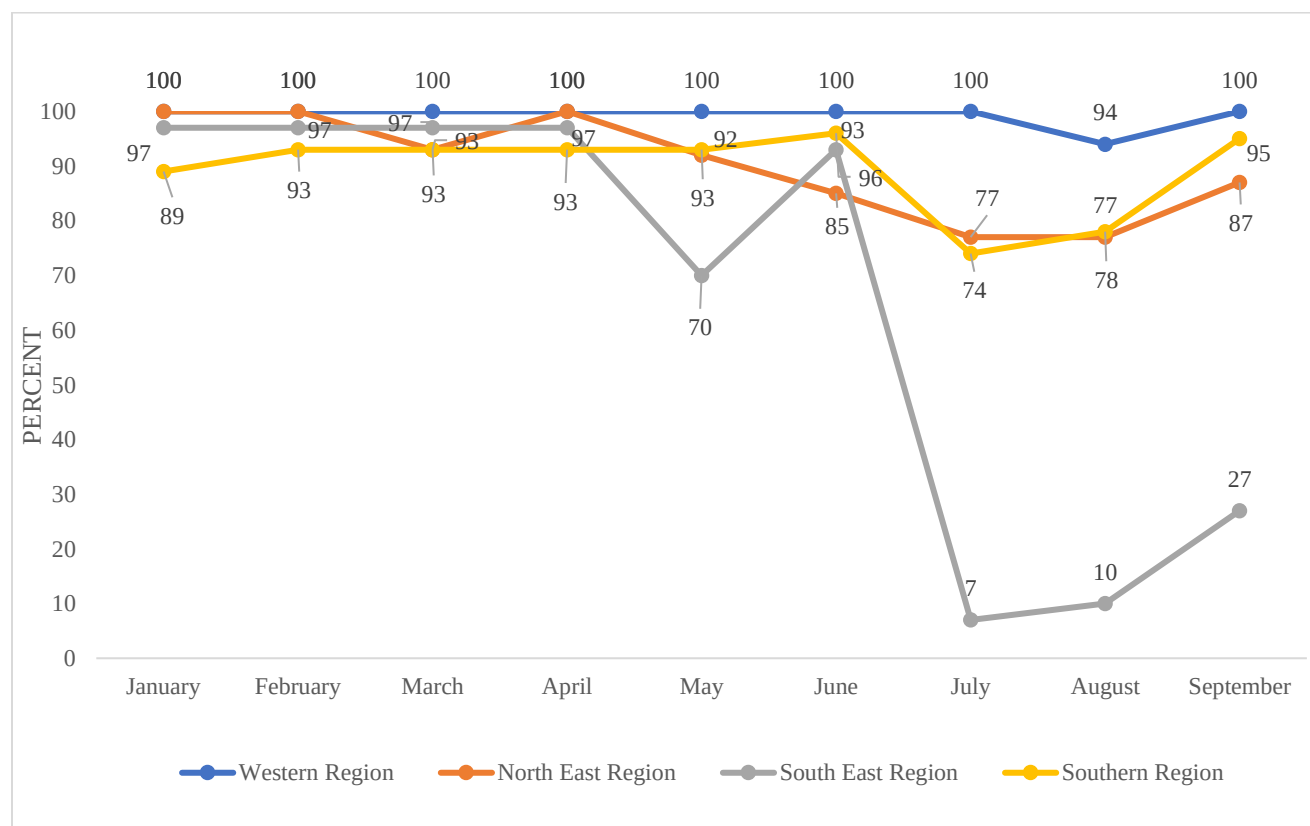
A total of 30,136 STI cases were recorded as at September, with 7,756 of those cases observed in the July to September quarter, of which females accounted for 78.9% of cases (6,120). The highest burden of disease was seen in the 20-29 and 30-39 age groups, followed by individuals aged 50+. Genital discharge was the most commonly reported syndrome, while genital herpes and syphilis continued to be reported, albeit at lower levels. Pelvic Inflammatory Disease and Epi treatment for GC/Chlamydia were the most reported Other STIs and Non-STI Conditions for the quarter. Epidemiological treatment of partners was notably low for syphilis. HIV and syphilis testing coverage among female clinic attendees was significantly lower than among males, despite females comprising the majority of attendees.

These indicators enable continuous monitoring of disease patterns and provide critical insights into the effectiveness of prevention and treatment interventions nationwide. In addition to national-level surveillance, the system supports the identification of high-burden populations, which is vital for targeting interventions and improving programme outcomes. Ongoing efforts to strengthen data quality, completeness, and timeliness remain central to ensuring a robust and reliable STI surveillance system.

Reporting Compliance

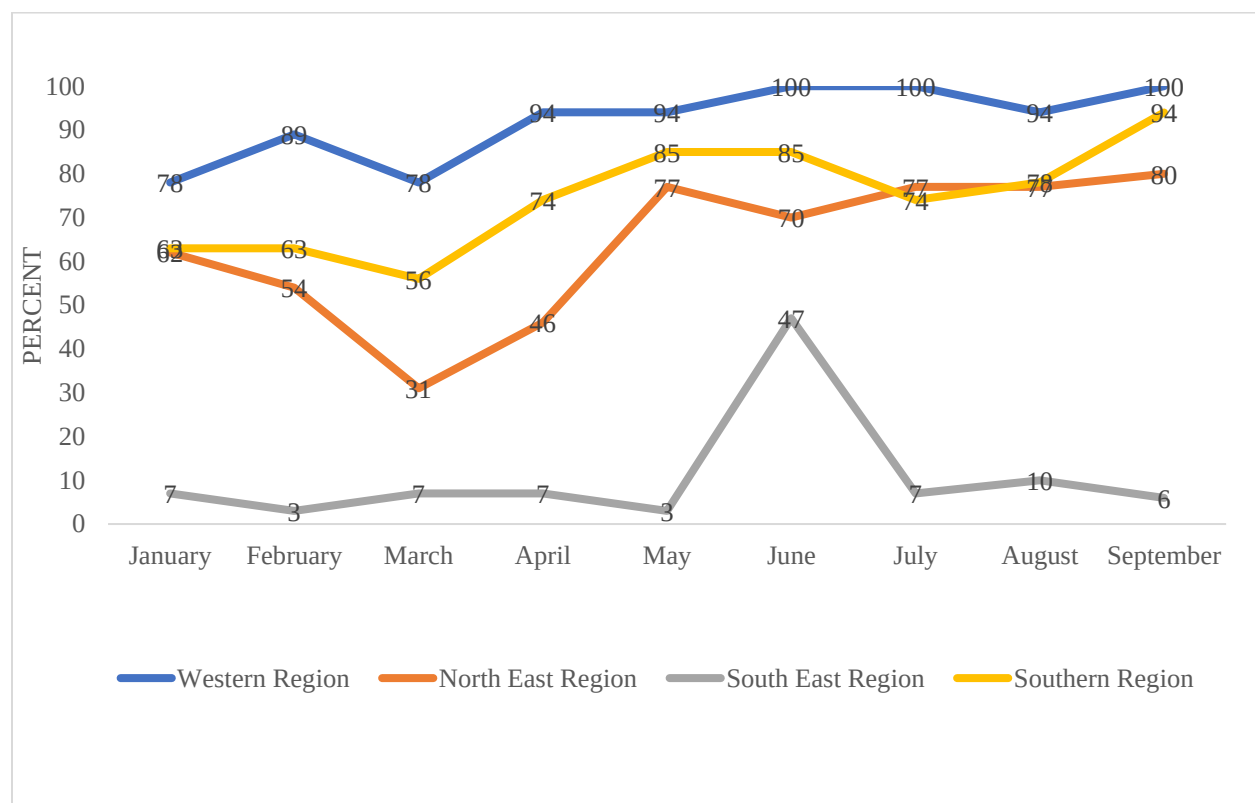
During the third quarter (July to September), the completeness of submitted reports shows significant variation across the regions. The Western Region maintains consistently strong performance, achieving 100% completeness in both July and September, with only a slight dip to 94% in August. The North East Region also performs well overall, though it experiences a noticeable drop in August to 77% before sharply improving to 95% in September, suggesting successful corrective actions. The Southern Region shows steady progress throughout the quarter, starting at 74% in July, rising to 78% in August, and further improving to 87% in September, indicating gradual strengthening of reporting compliance. In contrast, the South East Region faces severe challenges during this period, with completeness decreasing to 7% in July and only improving slightly to 10% in August and 27% in September. Although there is some recovery, performance remains significantly below expected standards (Figure 1.1).

Figure 1.1 Completeness of Submitted Reports by Region



From July to September, the timeliness of submitted reports shows clear regional differences, with some regions demonstrating strong consistency while others face ongoing challenges. The Western Region maintains excellent performance throughout the quarter, recording 100% in July, a slight dip to 94% in August, and returning to 100% in September. The North East Region also performs relatively well, though with some fluctuations as timeliness rises from 69% in July to 77% in August and improves further to 80% in September reflecting steady recovery and strengthened adherence to reporting timelines. The Southern Region demonstrates consistent improvement across the quarter, starting at 74% in July, increasing to 78% in August, and reaching 94% in September. In contrast, the South East Region continues to struggle significantly, with very low timeliness levels with 7% in July, a modest increase to 10% in August, and a slight decline to 6% in September. Despite brief improvement and while there are little to no data discrepancies observed, performance remains critically low (Figure 1.2).

Figure 1.2 Timeliness of Submitted Reports by Region

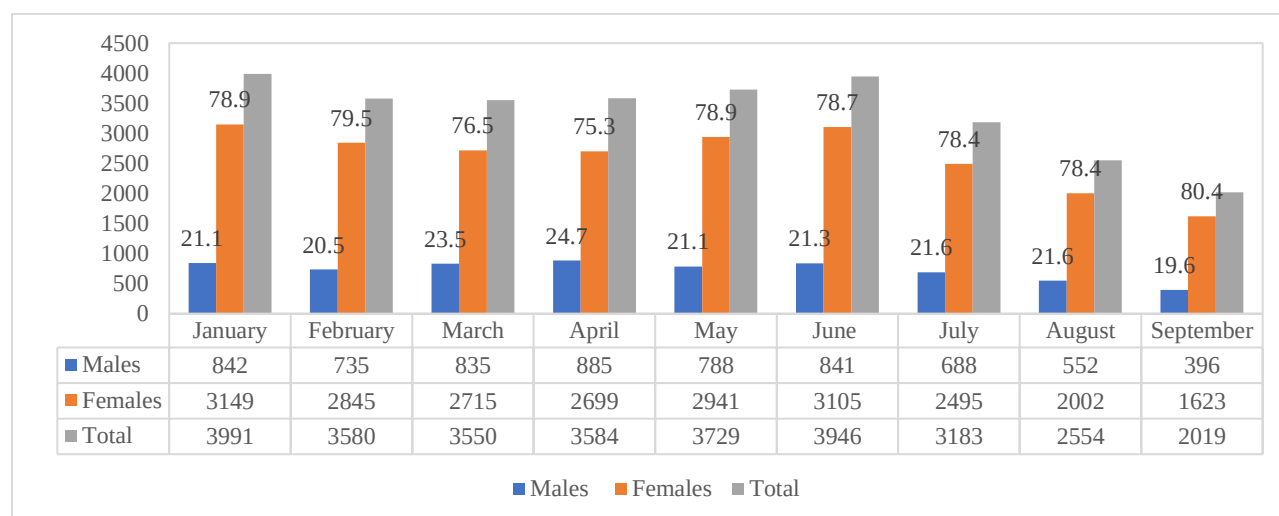


STI Surveillance

Gender

A total of 30,136 STI cases were recorded as at September 2025. There were 7,756 sexually transmitted infection (STI) cases reported during the July to September quarter, with 79% occurring among females (6,120) compared to 21% males (1,636). Throughout the quarter, the proportion of male cases remained stable at 22% in July and August, before declining by 2% to 20% in September, indicating a downward trend. Conversely, female cases increased by 2% in September, reflecting greater uptake among females. Compared to earlier months, male cases in September were the lowest recorded for the year, while female cases reached their highest reporting rate (Figure 2.1).

Figure 2.1 Monthly Trend of STI Cases by Gender

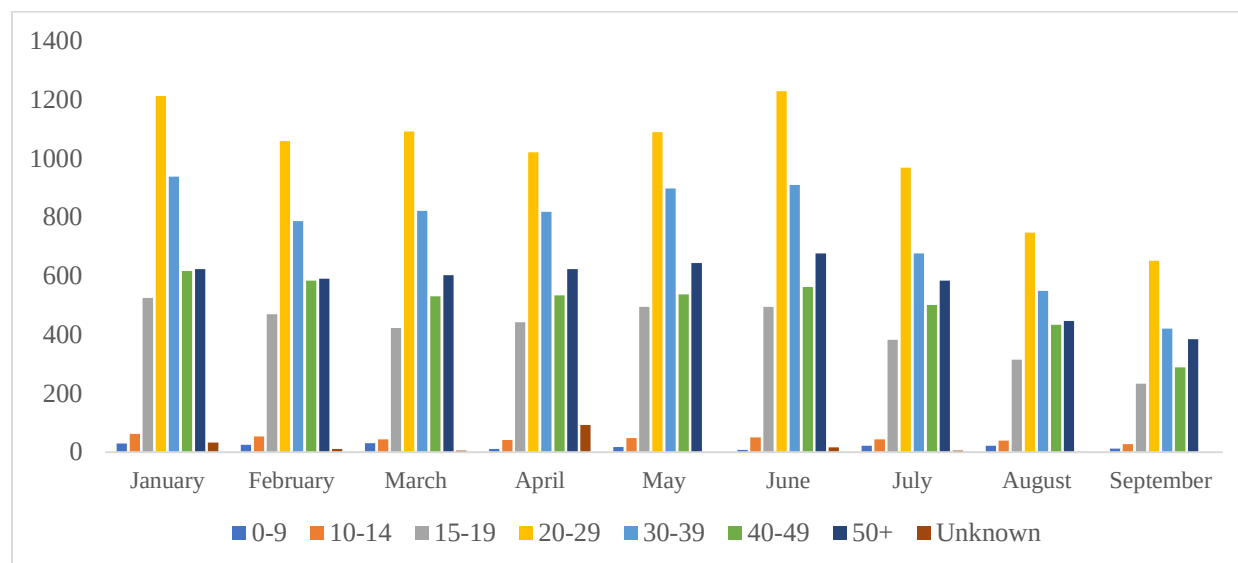


Age

The age distribution of STI cases from January to September reveals a clear and steady decline in STI cases across nearly all age groups. However, July reflects relatively high numbers, particularly among the 20–29 (969 cases) and 30–39 (676 cases) groups, but by August new cases fell significantly to 747 and 549, respectively. This downward trend continues into September, where the 20–29 group drops further to 652 cases and the 30–39 group to 421 cases which are the lowest monthly totals recorded for the year. Similar declines are seen among the 15–19 and 40–49 groups, both of which gradually decrease from July to September. The 50+ population also shows a steady reduction during this period, moving from 584 cases in July to 347 in September. It is observed that the July to September quarter

marks a consistent tapering of STI notifications across all major age groups, suggesting reduced testing and reporting (Figure 2.2).

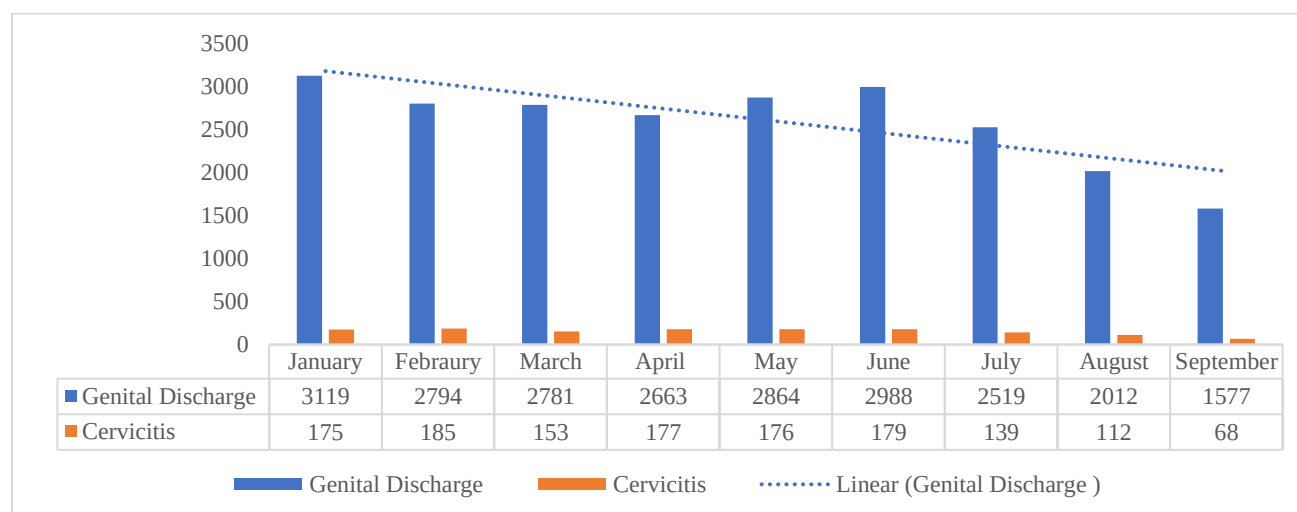
Figure 2.2 STI Cases Age Breakdown



Genital Discharge Syndrome

From July to September, both Genital Discharge and Cervicitis syndromes showed a steady decline, with Genital Discharge cases dropping from 2,519 in July to 1,577 in September, and Cervicitis cases decreasing from 139 to 68 over the same period, continuing the downward trend observed since the start of the year (Figure 2.3).

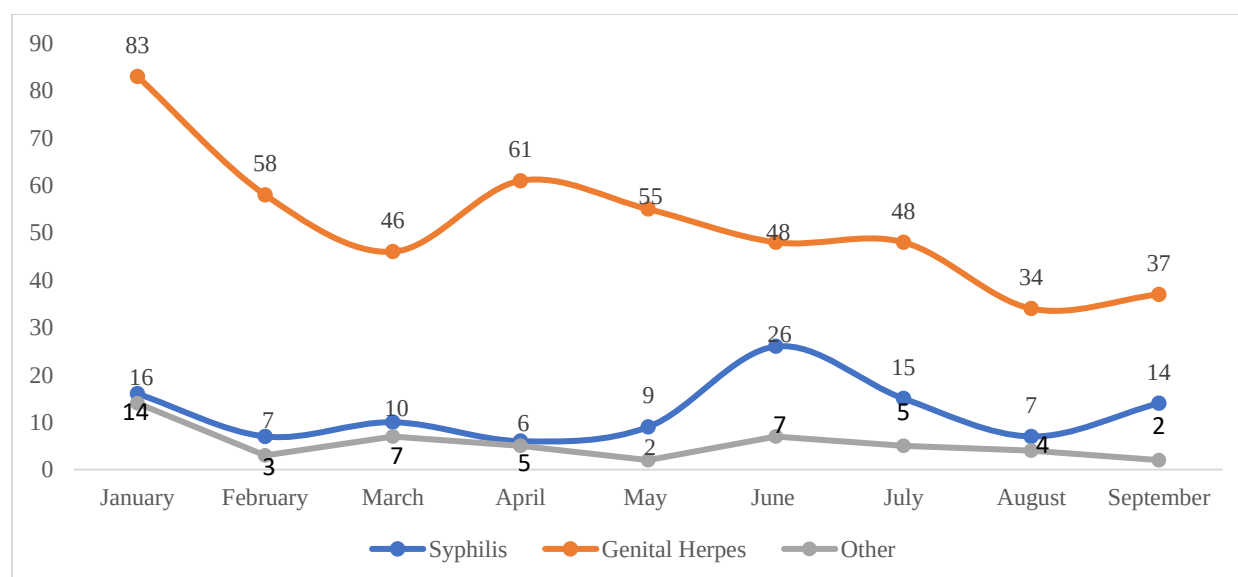
Figure 2.3 Genital Discharge Syndrome by Month



Genital Ulcer Disease

The July to September period reflects a general decline followed by mild resurgence for both Genital Herpes and Syphilis, while Other Ulcer Causes remain minimal. Genital Herpes shows notable fluctuations, holding steady at 48 cases in July before dropping sharply to 34 in August and then rising slightly to 37 in September. Syphilis declines from 15 cases in July to a low of 7 in August but increases again to 14 in September. Other causes remain consistently low throughout this period, decreasing from 5 cases in July to 4 in August and 2 in September (Figure 2.4).

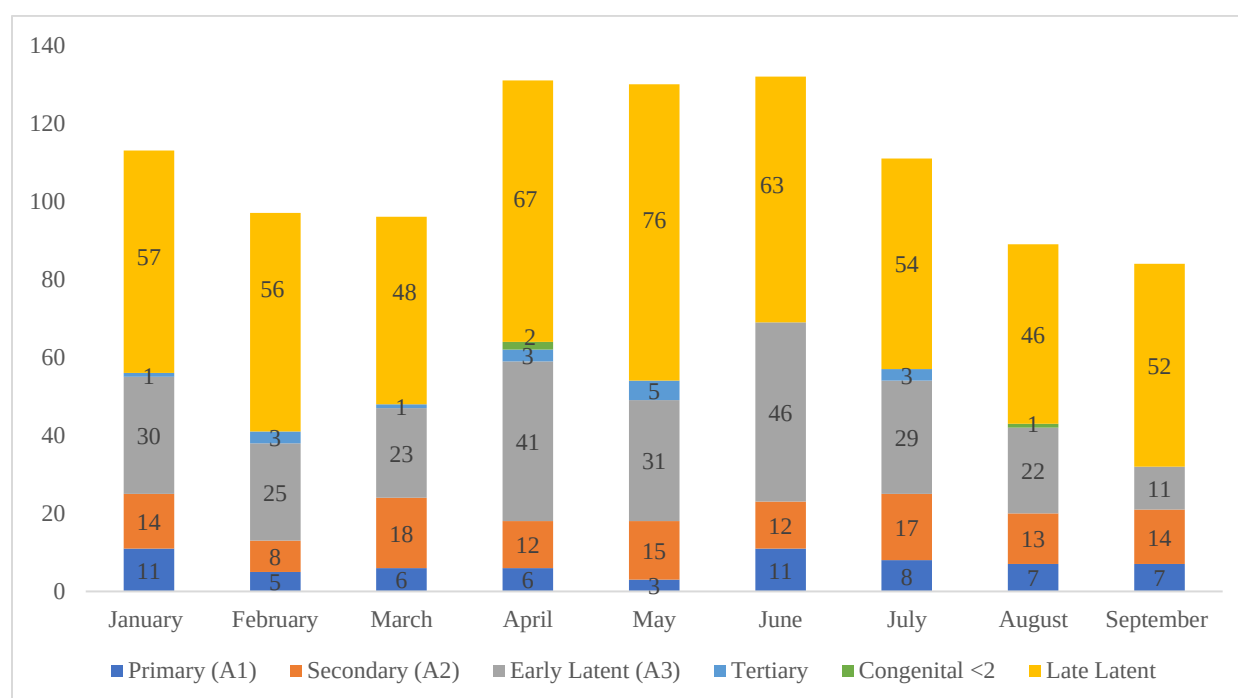
Figure 2.4 Genital Ulcer Disease Syndrome



Syphilis Cases

From July to September, syphilis cases show an overall decline, with July recording the highest total (111), followed by consistent reductions in August (89) and September (84). Early latent cases (A3) reflect this pattern, dropping steadily from 29 in July to 22 in August and then to 11 in September. Late latent infections decrease from 54 to 46 between July and August but slightly increased to 52 in September. Primary and secondary cases show only minor month-to-month shifts, while tertiary and congenital syphilis remain low throughout (Figure 2.5).

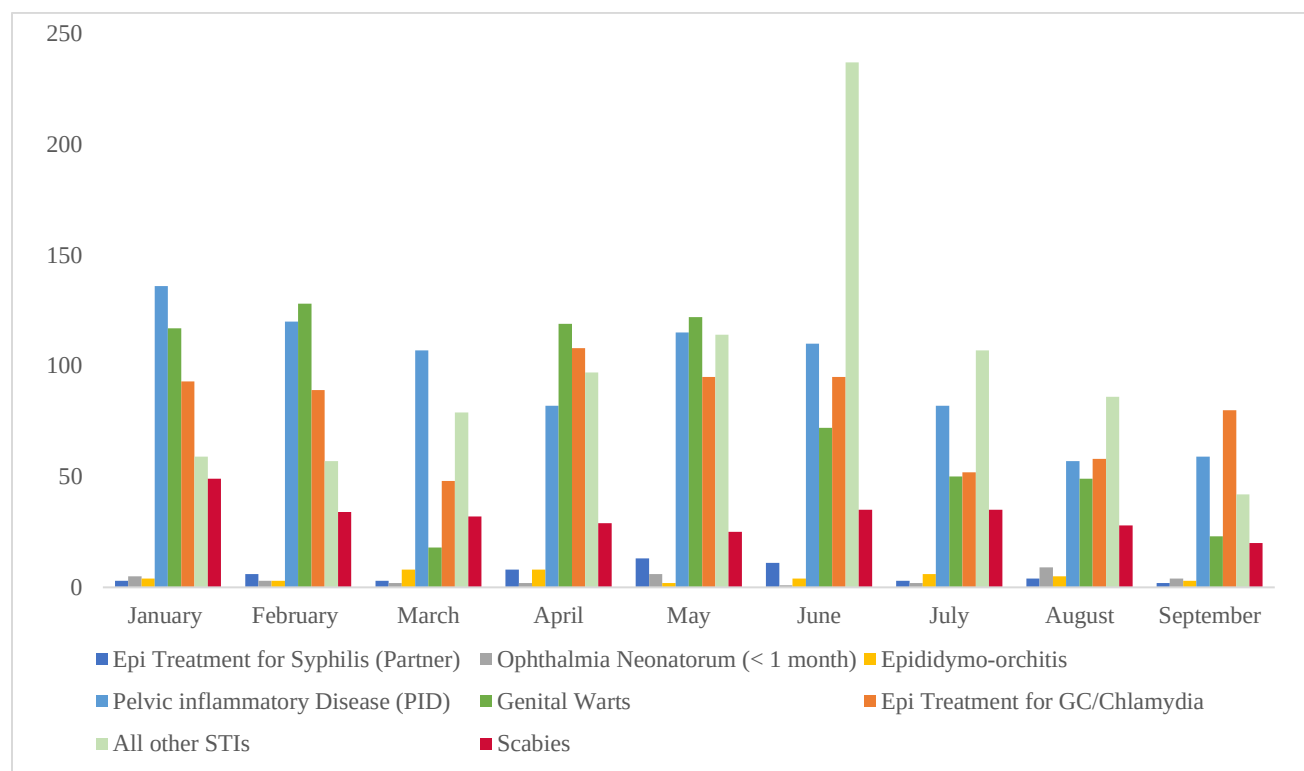
Figure 2.5 Syphilis Cases



Other STIs and Non-STI Conditions

From July to September, other STI and non-STI conditions exhibit notable fluctuations, with many declining in August. Pelvic Inflammatory Disease (PID) shows the sharpest movement, falling from 82 cases in July to a low of 57 in August, followed by a slight rise to 59 in September. Genital warts remain relatively stable between July (50) and August (49) but then drop steeply to 23 in September. Epi treatment for GC/Chlamydia increases from 52 in July to 58 in August and then surges to 80 in September, the only condition showing a strong upward rebound. Scabies steadily declines across the three months, moving from 35 in July to 28 in August and further down to 20 in September. All other STIs follow a downward trajectory as well, decreasing from 107 cases in July to 86 in August and continuing to fall to 42 in September. The beginning of the quarter marked higher case numbers across categories; August reflects a marked decline, and September shows partial recovery in only Pelvic Inflammatory Disease and Epi treatment for GC/Chlamydia which may largely be attributed to an increase in the number of reports received (Figure 2.6).

Figure 2.6 Other STIs and Non-STI conditions



STI Clinic Testing and Surveillance

Table 1. STI Clinic Testing Summary (January- September 2025)

Indicators	South-East		Western		Southern		North-East		Total		
	M	F	M	F	M	F	M	F	M	F	Total
ST Clinic Attendees	3,202	9,680	1,428	4,981	815	2,586	760	3,421	6,205	20,668	26,873
Tested for HIV	2,386	4,963	784	1,970	638	1,646	563	1,549	4,371 (30.0%)	10,128 (70.0%)	14,499
Positivity Yield (HIV)	28 (1.2%)	51 (1.0%)	23 (2.9%)	24 (1.2%)	37 (5.8%)	42 (2.6%)	2 (0.4%)	3 (0.2%)	90 (2.1%)	120 (1.2%)	210/ 14,499 (1.4%)
Tested for Syphilis	2,376	4,894	708	1,786	584	1,592	557	1,510	4,225 (30.0%)	9,782 (70.0%)	14,007
Positivity Yield (Syphilis)	66 (2.8%)	47 (1.0%)	20 (2.8%)	9 (0.5%)	24 (4.1%)	28 (1.8%)	13 (2.3%)	12 (0.8%)	123 (3.0%)	96 (1.0%)	219/ 14,007 (1.6%)

During the reporting period, a total of 26,873 individuals attended STI clinics nationally. Of these, 6,205 were males and 20,668 were females. HIV testing was conducted for 14,499 attendees, representing an overall testing coverage of 54%. Females had a higher testing uptake, with 70% (10,128) of attendees tested for HIV, compared to 30% (4,371) of males. However, the HIV positivity yield was also higher among males at 2.1% (90 cases), compared to 1.2% (120 cases) among females. Overall, the national HIV positivity yield stood at 1.4% (210 positive cases).

For syphilis testing, 14,007 attendees were tested, representing 52% of the total. Similar to HIV testing, females had a higher syphilis testing rate at 70% (9,782) while only 30% (4,225) of males were tested. However, the syphilis positivity yield was significantly higher among males at 3% (123 cases) compared to 1% (96 cases) in females. This resulted in an overall syphilis positivity rate of 1.6% (219 cases). Notably, the Southern Region recorded comparatively higher positivity yields for both HIV and syphilis during the quarter, accounting for 34% (72) of all new HIV cases (37 males, 42 females) and 24% (52) of all new syphilis cases (24 males, 28 females) reported nationally.

Action Sheet

Table 2. Action Items to Address Discrepancies

Challenges Identified	Intervention	Timeline	Responsible Persons
Late Submissions	Implement automated reminders and follow-ups through DHIS2 to improve timeliness and completeness of report submissions.	January 2026	Sheldon Whorms
Data Timeliness and Completeness	Monitor and report on how promptly each parish/region enters and completes reports, encouraging accountability and timely submissions.	Ongoing	Shanille Brissett Contact Investigators
Data Quality	Integrate data quality checks at weekly and monthly intervals to identify gaps in compilation of data for reporting.	Ongoing	Contact Investigators